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TEST NAME: WHOLE BODY F 18 FDG PET-CT SCAN

AGE / SEX: 64 Y / F
MAX ID: VSLI.253744
DATE: 04-APR-2025

WHOLE BODY PET-CT SCAN

Brief clinical history: Patient is a follow up case of carcinoma bilateral ovary. Post NACT, post-surgery and HIPEC followed by 6 cycles of chemotherapy. Presented with recurrence. Post 6 cycles of chemotherapy, post HIPEC. On maintenance bevacizumab + etoposide. Rising CA- 125 level. Current PET-CT is being done for further evaluation.

Procedure: Whole body FDG PET scan was acquired from vertex to mid thigh in a whole-body PET-CT scanner (GE Discovery IQ), one hour after intravenous injection of 370 MBq of ^{18}F -FDG. High resolution, 16 slice, contrast CT scan was also obtained of the same area. Oral contrast was administered for bowel opacification, over 90 min, before the scan. A table dose of oral contrast was given just before the scan. Images were reconstructed using Q Clear algorithm and slices were reformatted into transaxial, coronal and sagittal views. Semi-quantitative estimation of FDG uptake was performed by calculating SUV_{max} value, corrected for dose administered and lean body mass (g/ml).

Findings:

Brain:

Normal physiological tracer distribution noted in the brain parenchyma. No focal lesion or abnormal uptake noted in the brain.

Note: All brain metastases may not be apparent on a PET/CT scan and a MRI can be performed where clinically indicated.

Head and neck:

Mucosal thickening with no FDG uptake is seen in right maxillary sinus - inflammatory

Normal physiologic uptake noted in the nasopharynx, oropharynx, hypopharynx and larynx. Parotid and bilateral submandibular glands demonstrate normal metabolic activity.

The thyroid gland shows homogenous pattern on CT. No abnormal FDG uptake is seen in the thyroid.

No significant cervical / supraclavicular lymphadenopathy noted.

Thorax:

The heart and the mediastinal vascular structures are well opacified with I/V contrast. The trachea and main bronchi appear normal.

Mild fibrotic changes seen bilaterally. Nodular infiltrates seen in right lung field – infective / inflammatory. Multiple irregular peripheral ground glass opacities with no FDG uptake are seen in posterior segment both lung upper lobes and both lung lower lobes – (unchanged since previous study) - post infective changes.

Mild to moderate right pleural effusion noted with associated increased FDG uptake SUV_{max} 1.6.

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11-2010-0058
Oct 29, 18 - Oct 28, 22
Since Oct 29, 2010

FDG avid SUVmax 2.3 Vs previous SUVmax 3.1 few subcentimetric to centimeter sized lymph nodes seen in paracardiac and right epiphrenic regions.

No significant FDG avid mediastinal lymphadenopathy noted.

Abdomen and pelvis:

The liver is normal in size and shape No focal intrahepatic lesion is noted. *The intrahepatic biliary radicles are mildly dilated.* The portal vein is normal.

The gall bladder is well distended with no abnormal FDG uptake.
No evidence of ascites noted.

 The pancreas and spleen appear normal on CT with no abnormal FDG uptake. Bilateral adrenal glands appear normal in size with no abnormal FDG uptake.

Both the kidneys appear normal in size, shape and attenuation. No scan evidence of calculus or hydronephrosis is noted.

The stomach, opacified small bowel and large bowel loops appear normal in caliber and fold pattern.

Evidence of surgery noted in anterior abdominal wall.

Note is made of FDG avid soft tissue thickening along the bowel loops prominently in the left iliac fossa underneath the anterior abdominal wall SUVmax 3.8 and small bowel / colonic loops in right iliac fossa with associated mild wall thickening these foci of increased FDG uptake persist on the delayed scan and are suspicious for serosal deposits along the bowel loops.

No enlarged lymphnode with increased FDG uptake are seen in abdominal region.

Uterus is not visualized – post hysterectomy status. No focal abnormal FDG avid lesion is seen at uterine bed / vaginal vault.

 Urinary bladder is well distended with no abnormal FDG uptake.

Musculoskeletal system:

Mild osteopenia seen in visualized bones

The visualized portion of the musculoskeletal system shows normal physiological tracer distribution.

IMPRESSION:

IN A FOLLOW UP CASE OF CARCINOMA BILATERAL OVARY. POST NACT, POST-SURGERY AND HIPEC FOLLOWED BY 6 CYCLES OF CHEMOTHERAPY. PRESENTED WITH RECURRENCE. POST 6 CYCLES OF CHEMOTHERAPY, POST HIPEC. ON MAINTENANCE BEVACIZUMAB + ETOPOSIDE. RISING CA- 125 LEVEL. PET/CT SCAN SHOWS:

- UTERUS IS NOT VISUALIZED – POST HYSTERECTOMY STATUS. NO FOCAL ABNORMAL FDG AVID LESION IS SEEN AT UTERINE BED / VAGINAL VAULT.
- MILD TO MODERATE RIGHT PLEURAL EFFUSION NOTED WITH ASSOCIATED INCREASED FDG UPTAKE – LIKELY DISEASE INVOLVEMENT.
- FDG AVID FEW SUBCENTIMETRIC TO CENTIMETER SIZED LYMPH NODES SEEN IN PARACARDIAC AND RIGHT EPIPHRENIC REGIONS - LIKELY DISEASE INVOLVEMENT.
- NOTE IS MADE OF FDG AVID SOFT TISSUE THICKENING ALONG THE BOWEL LOOPS PROMINENTLY IN THE LEFT ILIAC FOSSA UNDERNEATH THE ANTERIOR ABDOMINAL WALL AND SMALL BOWEL / COLONIC LOOPS IN RIGHT ILIAC FOSSA WITH ASSOCIATED MILD WALL THICKENING THESE FOCI OF INCREASED FDG UPTAKE PERSIST ON THE DELAYED SCAN AND ARE SUSPICIOUS FOR SEROSAL DEPOSITS ALONG THE BOWEL LOOPS.
- NO FOCAL ABNORMAL FDG AVID LESION IN REST OF THE BODY.

AS COMPARED TO PREVIOUS PET-CT SCAN DATED 29-11-2024, THERE IS MILD INCREASE IN RIGHT SIDE PLEURAL EFFUSION NOTED. RIGHT EPIPHRENIC AND PARACARDIAL LYMPH NODES SHOW DECREASE IN METABOLIC ACTIVITY. INTERVAL NEW DEVELOPMENT OF SUSPICIOUS METABOLICALLY ACTIVE SEROSAL DEPOSITS.

OVERALL SCAN FINDINGS FAVOR DISEASE PROGRESSION.


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